

Media Information

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Atlantic Medical Imaging - Synapse EIS

**Accurate answers. Fast.**

30 East Maryland Avenue, Somers Point, NJ 08244 • P: [REDACTED] • F: [REDACTED]

Name: [REDACTED]
MRN: 930411641
DOB: [REDACTED]
Acc #: 2074261676
Sec Acc#: 2074261676
Exam Date: 8/4/2025 10:42 AM

Address: 599 SHORE RD STE 101
[REDACTED]
Referrer Phone#: (609)926-8353
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cc
cc

DIGITAL BILATERAL DIAGNOSTIC MAMMOGRAM with CAD and 3D DIGITAL TOMOGRAPHY AND BILATERAL BREAST ULTRASOUND: 8/4/2025 9:08 AM

INDICATION: 83 years old presenting for further evaluation of increasing bilateral nipple inversion.

COMPARISON: Multiple prior mammograms, the most recent dated [REDACTED] though dating back to [REDACTED]
Breast MRIs dated [REDACTED] and [REDACTED]

PERSONAL HISTORY/RISK FACTORS: No [REDACTED]

FAMILY HISTORY OF BREAST CANCER: No [REDACTED]

RISK ASSESSMENT: The patient's lifetime breast cancer risk according to the Tyrer-Cuzick model is 7.7 %, (Based on a revised TC model (version 8) which now incorporates breast density).

DIAGNOSTIC MAMMOGRAM

TECHNIQUE: 2D and 3D combo digital tomography images were obtained. Standard images were reviewed with computer-aided detection.

BREAST DENSITY: The breasts are heterogeneously dense, which may obscure small masses (Category C).

FINDINGS

There is an asymmetry superiorly on true lateral view at posterior depth, localizing to the lateral breast (true lateral slice 15/59), which persists with spot compression (true lateral spot compression slice 17/49). Elsewhere in either breast, there are stable mammographic findings including oil cysts and coarse calcifications. No suspicious finding is identified in the contralateral left breast.

BILATERAL ULTRASOUND

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Targeted sonographic images of the bilateral retroareolar regions due to patient's concern of increasing nipple inversion and right upper outer quadrant in the area of questioned mammogram finding were obtained. In the bilateral retroareolar regions there is no [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] [REDACTED] bilateral nipple inversion. At 10:00, 10 cm from the nipple there is an irregular hypoechoic mass measuring 0.5 x 0.2 x 0.3 cm. This finding corresponds to a mammographic asymmetry. Survey ultrasound of the right axilla shows normal morphology lymph nodes.

IMPRESSION

(D). Right breast 0.5 cm irregular hypoechoic mass at 10:00, 10 cm from the nipple, corresponding to a mammographic asymmetry. Ultrasound-guided core needle biopsy is recommended.

{0}.No right axillary adenopathy.

(0). No suspicious mammographic or sonographic abnormality in the bilateral retroareolar regions to explain patient's bilateral increasing nipple inversion.

{0}.No-suspicious mammographic finding in the left breast.

BI-RADS 4B: Suspicious Finding

RECOMMENDATION

Right breast ultrasound guided core needle biopsy should be performed in the absence of clinical contraindications. Recommend this ultrasound-guided core needle biopsy is performed prior to recommended short-term follow-up breast MRI.

These findings and recommendations were discussed with the patient in person, giving her the opportunity to answer questions and acknowledge understanding.

In accordance with New Jersey law, the following statement must be included in all mammography reports regardless of an individual's breast tissue density:

"Your mammogram may show that you have dense breast tissue as determined by the Breast Imaging Reporting and Data System established by the American College of Radiology. Dense breast tissue is very common and is not abnormal. However, in some cases, dense breast tissue can make it harder to find cancer on a mammogram and may also be associated with a risk factor for breast cancer. Discuss this and other risks for breast cancer that pertain to your personal medical history with your health care provider. A report of your results was sent to your health care provider. [REDACTED] density at the website of the American College of Radiology, www.acr.org."

If you are the referring physician [redacted] to reach a radiologist.

Date Of Signature 8/4/2025 1:07 PM

Report Electronically Signed on: 8/4/2025 01:07 PM

Patient [REDACTED] [REDACTED] [REDACTED]

Patient [REDACTED] [REDACTED]

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Document Information

Misc Administration: Outside Radiology, Breast MAMMO

00:00

Attached To:

OUTSIDE RADIOLOGY ORDER

Orders Only on with Historical Provider,

Source Information

Priscilla P. Hazel | Msk Pas